RIVERTON GENERAL HOSPITAL (FICTIONAL) - DISCHARGE SUMMARY
SYNTHETIC TEST DOCUMENT - NOT A REAL PATIENT
Patient: SAMPLEWOOD, Jennifer Ann MRN: SYN-0042817
DOB: 19/07/1983 Sex: Female
Admitted: 21/09/2026 Discharged: 25/09/2026
Consultant: Dr T. Example, Respiratory Medicine

Diagnoses
1. Community-acquired pneumonia, right lower lobe (J18.1)
2. Asthma, moderate persistent (J45.9)
3. Hypothyroidism (E03.9)
Past history: gestational diabetes 2014, resolved postpartum. Appendicectomy 2001.

Allergies
NKDA. Shellfish allergy - throat swelling and hives (2019).

Hospital course
Admitted with fever, productive cough and SpO2 89% on air. CXR: RLL consolidation.
Treated with IV antibiotics for 3 days, then switched to oral amoxicillin.
Salbutamol nebs PRN on the ward. Montelukast was stopped on admission (no benefit) and will not be restarted. Continued on her usual levothyroxine and Symbicort.

Discharge medications

Amoxicillin 500 mg capsules, 1 capsule PO TDS for 4 more days (course ends 29/09/2026)
Levothyroxine 75 micrograms PO OD
Symbicort 200/6 two puffs BID
Salbutamol 100 mcg inhaler 2 puffs PRN for wheeze

Stopped medications

Montelukast 10 mg nocte - stopped 21/09/2026 (no benefit)

Follow-up

GP review in 1 week. Repeat CXR in 6 weeks.
Dr T. Example